

Correct answer: d. Likely related to an allergic response

Rationale:

Timing + itching + raised rash after detergent = contact dermatitis.

Question 22

A client with a history of chronic obstructive pulmonary disease (COPD) presents with clubbed nails. The

Nurse understands that this finding is MOST directly related to:

- a. Inadequate grooming due to fatigue.b. a concurrent fungal infection.
- b. Protein deficiency from poor nutrition.
- c. Long-term oxygen deprivation.

Correct answer: d. Long-term oxygen deprivation

Rationale:

Clubbing results from chronic hypoxia, common in COPD.

Question 23

Which symptom in the health history is defined as a burning sensation in the esophagus due to gastric acid reflux?

- a. Hematemesis

- b. Pyrosis
- c. Melena
- d. Eructation

Correct answer: b. Pyrosis

Rationale:

Pyrosis = heartburn caused by gastric acid reflux.

Question 24

When evaluating the statement, “It may be difficult to palpate the apical impulse in clients who are obese,” the MOST accurate rationale is that in these clients:

- a. The Point of Maximal Impulse (PMI) shifts to the 4th intercostal space.
- b. The distance from the apex of the heart to the precordium is increased.
- c. The heart is rotated anteriorly.
- d. There is decreased adipose tissue over the precordium.

Correct answer: b. Increased distance from heart apex to chest wall

Rationale:

Obesity increases adipose tissue, making PMI difficult to palpate.

Question 25

Why is stridor considered a medical emergency? It:

- a. Is associated with chronic bronchitis.
- b. Is a sign of lower lobe atelectasis.
- c. Indicates fluid in the small airways.
- d. Signals an upper airway obstruction.

Correct answer: d. Signals an upper airway obstruction

Rationale:

Stridor is a high-pitched emergency sound indicating airway compromise.

Question 26

A patient with a history of COPD is likely to exhibit which of the following findings during inspection?

- a. Increased diaphragmatic excursion
- b. A decreased AP diameter

- c. An increased AP diameter (barrel chest)
- d. Tracheal deviation to the left

Correct answer: c. Increased AP diameter (barrel chest)

Rationale:

Classic COPD finding due to hyperinflation.

Question 27

A patient denies health problems yet exhibits jaundice and abdominal distension.
Which action reflects

Critical evaluation of assessment data?

- a. Document contradiction but make no clinical judgment
- b. Prioritize objective findings indicating possible liver disease
- c. Delay physical examination to avoid patient distress
- d. Accept patient's denial since he is alert

Correct answer: b. Prioritize objective findings

Rationale:

Objective data overrides denial when serious pathology is suspected.

Question 28

Which source of health information is considered primary data in health history taking?

- a. The spouse explaining the client's symptoms
- b. A mentally alert patient describing his sensations
- c. A nurse's measurement of vital signs
- d. A previously documented medical record

Correct answer: b. A mentally alert patient describing his sensations

Rationale:

Primary data comes directly from the patient.

Question 29

Which of the following is defined as the displacement of the urinary meatus to the ventral surface of the

Penis?

- a. Epispadias

- b. Phimosis
- c. Cryptorchidism
- d. Hypospadias

Correct answer: d. Hypospadias

Rationale:

Hypospadias = urethral opening on ventral surface of penis.

Question 30

Why should a nurse never palpate both carotid arteries simultaneously? It can:

- a. Dislodge carotid plaque and cause embolism.
- b. Make comparing pulses slightly difficult.
- c. Reduce cerebral flow and trigger syncope.
- d. Create notable discomfort for the patient.

Correct answer: c. Reduce cerebral flow and trigger syncope

Rationale:

Palpating both carotids can decrease brain perfusion.

Question 31

A nurse observes a roughening and darkening of the skin on a client's posterior neck. This finding is MOST consistent with:

- a. Cyanosis, signaling a cardiopulmonary problem.
- b. Acanthosis nigricans, suggesting possible diabetes mellitus.
- c. Jaundice, associated with hepatic dysfunction.
- d. Vitiligo, indicating an autoimmune condition.

Correct answer: b. Acanthosis nigricans

Rationale:

Dark, velvety neck skin suggests insulin resistance/diabetes.

Question 32

During inspection, a bulge of red mucous membrane appears at the anal opening when the client strains.

The nurse analyzes this as MOST consistent with:

- a. Poor sphincter tone.

b. A thrombosed external hemorrhoid.

c. A perianal abscess.

d. Rectal prolapse.

Correct answer: d. Rectal prolapse

Rationale:

Red mucosa protruding during straining = rectal prolapse.

Question 33

When assessing a patient's carotid artery, a nurse palpates a weak and thready pulse.
The nurse should

Suspect a problem with:

a. The patient's pulmonary valve. b. Left ventricular stroke volume.

b. The patient's metabolic rate.

c. Right ventricular contraction.

Correct answer: b. Left ventricular stroke volume

Rationale:

Carotid pulse reflects left ventricular output.

Question 34

What is the primary cause of a carotid bruit?

- a. Turbulent blood flow through a partially blocked artery
- b. Hypertension causing pressure changes in the artery
- c. Increased cardiac output through a major artery
- d. Inflammation within a narrowed carotid artery

Correct answer: a. Turbulent blood flow through a partially blocked artery

Rationale:

Bruits are caused by turbulence from arterial narrowing.

Question 35

A patient presents with acute shortness of breath. On assessment, the nurse finds markedly absent breath

Sounds and no tactile fremitus on the right side. The trachea is deviated to the left. These collective findings are MOST consistent with:

- a. A large pleural effusion on the right side, which would push the trachea to the opposite side.
- b. Widespread bronchospasm from asthma, which would cause wheezes throughout all lung fields.
- c. Lobar pneumonia on the right side, which would increase fremitus and cause equal expansion.
- d. A tension pneumothorax on the right side, which would push the trachea to the opposite side.

Correct answer: d. Tension pneumothorax

Rationale:

Absent sounds + no fremitus + tracheal deviation = tension pneumothorax.

Question 36

During auscultation, a nurse hears continuous, low-pitched, snoring sounds over the main bronchi. These

Sounds clear after the patient coughs. The nurse should differentiate this as which type of sound?

- a. Wheezes
- b. Stridor

c. Rhonchi

d. Crackles

Correct answer: c. Rhonchi

Rationale:

Low-pitched, snoring sounds that clear with cough = rhonchi.

Question 37

Percussion over normal lung tissue produces which of the following sounds?

a. Resonance

b. Tympany

c. Hyper-resonance

d. Dullness

Correct answer: a. Resonance

Rationale:

Normal lung percussion produces resonance.

Question 38

During an admission assessment, the nurse notes a bluish discoloration of the client's lips and tongue.

Peripheral areas are also cool. Which term MOST accurately describes the discoloration of the lips and tongue?

- a. Mottling
- b. Central cyanosis
- c. Peripheral cyanosis
- d. Pallor

Correct answer: b. Central cyanosis

Rationale:

Bluish lips and tongue reflect central oxygen desaturation.

Question 39

During auscultation, a nurse hears bibasilar crackles (rales) in a patient with a history of heart failure. The nurse recognizes that these sounds are caused by:

- a. A harsh, high-pitched upper airway obstruction requiring emergency intervention.
- b. Secretions in the large airways that may clear with coughing or suctioning.

- c. Fluid in the small airways, suggesting the presence of pulmonary edema.
- d. The narrowing of airways from bronchospasm, as seen in anaphylaxis.

Correct answer: c. Fluid in the small airways

Rationale:

Bibasilar crackles in heart failure = pulmonary edema.

Question 40

During deep palpation of the LUQ, the nurse feels a mass with a sharp edge and a palpable notch. The

Nurse concludes that this finding is MOST consistent with which of the following?

- a. A gastric tumor
- b. An enlarged kidneyc. An enlarged spleen
- c. A hepatic tumor

Correct answer: c. Enlarged spleen

Rationale:

LUQ mass with notch is classic for splenomegaly.

Question 41

While assessing a patient's posterior chest expansion, the nurse places their thumbs at the level of the 10th ribs and asks the patient to take a deep breath. The nurse observes that the thumbs move apart less than 2 cm and asymmetrically. This finding is MOST indicative of which of the following?

- a. Decreased diaphragmatic function, as commonly seen in COPD.
- b. A normal and expected chest excursion for an adult patient.
- c. Hyperinflation of the lungs due to an acute asthma attack.
- d. A well-conditioned respiratory system with strong muscle use.

Correct answer: a. Decreased diaphragmatic function

Rationale:

Reduced, asymmetric expansion suggests restricted lung movement.

Question 42

The palpable vibrations transmitted through the bronchopulmonary tree to the chest wall are called:

- a. Resonance
- b. Rhonchi